

AETHERA · SPECIALTY BILLING SERIES

Family Medicine Billing Solutions

Maximizing Revenue Across Office E/M, Preventive Care, Chronic Care
Management, Telehealth, and Wellness

Prepared by: Aethera Healthcare Solutions · Lakeland, FL

Scope: CPT 99202–99215 · 99381–99397 · 99490/99439/99487 · G0438/G0439 (AWV) · 90471–90480
(vaccines)

Updated: 2026 Edition · aetherahealthcare.com · +1 (863) 694-0325

CCM · BHI · TCMAWV · Preventive Telehealth · Modifier 95 Vaccines · NDC



Primary Care Revenue, Engineered

2026 RCM EXECUTIV



ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served
Across 25+ specialties

\$12M+

Revenue Recovered
For clients in 2024–25

95%

Clean Claim Rate
vs 84% industry avg

< 30d

Average AR
Days in receivables



Personal, Not Corporate

Dedicated account team with deep specialty and payer knowledge. Same faces, same names — every cycle.



Performance Targets, In Writing

96%+ net collection · <5% denial · 95%+ coding accuracy
· 24h posting · 48h submission · 65%+ appeal success.



Zero Risk To Start

No setup fees. No long-term contracts. 90-day cancel.
Free 5-day audit before you commit.



FAMILY MEDICINE · MARKET CONTEXT

Why Family Medicine Billing Matters Now

Family medicine is the largest primary care specialty — 250M+ Americans have a primary care provider and the category represents \$285B in annual treatment spend. Volume-driven revenue means small per-claim optimization compounds quickly.

\$285B

**US primary care treatment spend
(2024)**

Largest single category of US medical spend.

250M+

**Americans with a primary care
provider**

Family medicine handles ~40% of all US office visits.

~ 10%

Family medicine claim denial rate

Lower than specialty avg, but high absolute volume drives rework cost.

\$1,000+

**Annual Medicare AWV + CCM
opportunity per eligible patient**

AWV (\$185+\$95) + CCM (\$110/yr) + BHI — most practices capture <30%.

Net effect: family medicine practices that enroll patients in CCM, complete Annual Wellness Visits, and enforce telehealth modifier discipline recover **8–15% of total revenue** currently lost to under-billing and denials.

FAMILY MEDICINE · OVERVIEW

The Four Pillars of Family Medicine Billing

Each pillar represents a distinct service family with unique coding, modifier, and documentation requirements.



01 · E/M +

Office E/M & Acute Care

Office visits (99202–99215), prolonged services (99417), telehealth with modifier 95 + POS 02. Volume-driven revenue; 2021 E/M reform shifted documentation to MDM-based.

CPT 99202–99215 · modifier
95 · POS 02



03 · CCM + B†

Chronic Care Management

CCM (99490/99439/99487), BHI (99484/99492–99494), TCM (99495/99496). Monthly recurring revenue per eligible patient — most family medicine patients qualify (≥2 chronic conditions).

CPT 99490 · 99439 · 99495 ·
99496



02 · PREVENTIVE +

Preventive Care & Wellness

Well visits (99381–99397) by age band, Annual Wellness Visit (G0438 initial, G0439 subsequent), sports physicals (99212–99215 with appropriate dx). HRA + cognitive + fall risk required for AWV.

CPT 99381–99397 · G0438
· G0439



04 · PROCEDURES +

Procedures & Vaccines

Vaccines (90471–90480 admin + vaccine product codes), minor procedures (17000–17250 destruction, 11102–11107 biopsy), joint injections (20600–20610), IUD insertion (58300). NDC required on all vaccine claims.

CPT 90471 · 20610 · 11102 ·
58300



PILLAR 01 · E/M + ACUTE

Office Visits, Telehealth & Acute Care

Family medicine office encounters cover acute visits (URIs, injuries, urgent care), chronic disease follow-up, and medication management. The 2021 E/M coding reform shifted documentation to MDM-based or time-based. Telehealth is now a permanent revenue stream — but modifier 95 + POS 02 discipline is required for parity reimbursement.

E/M coding (2021+ reform)

Code by Medical Decision Making complexity or time; 99202–99215. Modifier 25 required if same-day procedure.

Telehealth (post-PHE)

Modifier 95 + POS 02 for parity reimbursement. Audio-only allowed for mental/behavioral health with appropriate modifiers.

Prolonged services

992XX prolonged E/M without direct contact; 99417 prolonged service add-on (15-min increments).

Telehealth parity reimbursement (modifier 95 + POS 02) is the single most under-captured family medicine revenue line — most practices bill at the non-parity rate by default.

CPT Mini-Table — Top Office E/M Codes



Code	Service	Avg \$
99213	Est. E/M, low MDM	\$92
99214	Est. E/M, moderate MDM	\$131
99215	Est. E/M, high MDM	\$187
99417	Prolonged service add-on, 15 min	\$90
99202	New patient E/M, straightforward	\$80
99203	New patient E/M, low MDM	\$113



PILLAR 02 · PREVENTIVE + WELLNESS

Annual Wellness Visits, Preventive Exams & Sports Physicals

Preventive care is the highest-margin family medicine service line — fixed-time visits with documented requirements and minimal PA friction. AWV (G0438/G0439) is the single most under-completed Medicare service.

Medicare Annual Wellness Visit (AWV)

G0438 initial AWV (\$185) — requires HRA + fall risk + cognitive assessment. G0439 subsequent AWV (\$95). Patient owes \$0. Most practices complete <35% of eligible patients.

Preventive Medicine Visits

99381–99387 new patient; 99391–99397 established. 99395 (18–39, \$185), 99396 (40–64, \$215), 99397 (≥65, \$235). Includes age-appropriate screening + counseling.

Sports & School Physicals

99212–99215 with appropriate dx (Z02.0 sports, Z02.8 school). NOT covered by insurance — cash-pay \$50–\$150 typical. Document clearly to avoid denied insurance claim.

Counseling & Risk Reduction

99401–99404 preventive counseling (15–60 min); 99406–99407 tobacco cessation; 99411–99412 group counseling. Separate from E/M — bill by time.

✓ AWV Documentation Checklist

Health Risk Assessment (HRA), fall risk screening, cognitive assessment, depression screening (PHQ-9), functional status, list of providers/risk factors, personalized 5-yr screening plan, advance care planning (optional 99497).

⚠ Preventive + Problem E/M Same Day

Both billable with modifier 25 on the problem E/M. Common scenario: AWV + hypertension management. Without modifier 25, the problem E/M is denied.

↗ Top Capture Opportunity

70% of family medicine patients are AWV-eligible; most practices complete <35%. Lifting to 75% on a 1,000-patient Medicare panel adds ~\$185K/year in AWV revenue alone.

PILLAR 03 · CCM + BHI + TCM

CCM, Behavioral Health Integration & Transitional Care Mgmt

Chronic Care Management is the highest-margin recurring revenue stream in family medicine. Most family medicine patients qualify (≥2 chronic conditions expected to last 12+ months). CCM, BHI, and TCM are billed monthly/per-episode with relatively low documentation burden — but require an electronic care plan and 24/7 access.

Chronic Care Management (CCM)

99490 first 20 min/month (\$62.60); 99439 add'l 20 min (\$47.40); 99487 complex 60 min (\$100). Requires ≥2 chronic conditions + electronic care plan + 24/7 access.

Behavioral Health Integration (BHI)

99484 initial BHI (\$48); 99492 first 70 min/month (\$48); 99493 add'l 30 min (\$26); 99494 add'l 30 min (\$26). Behavioral health care manager required.

Transitional Care Management (TCM)

99495 moderate complexity, 14-day follow-up (\$190); 99496 high complexity, 7-day follow-up (\$280). Billed once per 30-day post-discharge window.

CCM is the single most under-billed code in family medicine — most eligible patients are not enrolled. A 1,000-patient CCM panel generates \$420K+/year of recurring monthly revenue.

Recurring Revenue Stacked Stats



\$62.60

99490 CCM first 20 min/month reimbursement

\$420K/yr

1,000-patient CCM panel annual revenue

\$280

99496 TCM high-complexity per episode

\$1,000+

Combined CCM + BHI + TCM per eligible patient per year

PILLAR 04 · PROCEDURES + VACCINES

Vaccines, Joint Injections, Minor Procedures & IUD

Procedures and vaccines drive ancillary revenue in family medicine. Vaccine billing requires both administration code (90471–90480) AND product code (90630, 90686, etc.) plus NDC. Joint injections require laterality documentation.

Vaccines & Immunizations

90471 admin first vaccine (\$25); 90472 admin add'l (\$16). Product codes: 90630 flu, 90686 flu HD, 90714 Tdap, 90389 RSV, 90670 Prevnar. NDC required on every vaccine claim. Modifier 25 if same-day E/M.

Joint & Soft Tissue Injections

20600 small joint (\$58); 20605 intermediate joint (\$78); 20610 major joint (\$108). Document laterality (LT/RT) and steroid (J1020, J1100, J3301). US guidance 76942 add-on.

Skin Procedures

17000 destruction premalignant lesion, first (\$120); 17003 add'l (\$30); 17110 benign destruction, up to 14 (\$140); 11102 tangential biopsy, single (\$135).

IUD & Women's Health

58300 IUD insertion (\$230); 58301 IUD removal (\$180); 57170 diaphragm fitting (\$95). Often covered without PA; document contraceptive counseling.

Procedure Reimbursement



\$25 + \$16

Vaccine admin first (90471) + add'l (90472)

\$108

Major joint injection (20610) — knee, shoulder

\$230

IUD insertion (58300)

NDC + Laterality Discipline

Every vaccine claim must include the 11-digit NDC alongside the product CPT code. Missing NDC = auto-denial on Medicare and most commercial plans. Joint injections must also document laterality (LT/RT modifier) — missing laterality = denial.

FAMILY MEDICINE · BILLING CODES

Top CPT/HCPCS Codes & 2026 Reimbursement

Most-billed codes across the four family medicine pillars.

Code	Service	Modifier	Avg \$	Notes
99213	Est. E/M, low MDM	—	\$92	Most common FM E/M
99214	Est. E/M, moderate MDM	—	\$131	Chronic disease management
99215	Est. E/M, high MDM	—	\$187	Complex / 40-min visits
99395	Preventive, 18–39 yrs	—	\$185	Established patient
99396	Preventive, 40–64 yrs	—	\$215	Established patient
99397	Preventive, ≥65 yrs	—	\$235	Established patient
G0438	Initial Annual Wellness Visit	—	\$185	HRA + fall + cognitive required
G0439	Subsequent AWV	—	\$95	Annual thereafter
99490	CCM, first 20 min/month	—	\$62.60	Monthly recurring revenue
99439	CCM, add'l 20 min/month	—	\$47.40	Max 2 add'l per month
99495	TCM, moderate, 14-day f/u	—	\$190	Post-discharge episode
Modifier 25 — significant, separately identifiable E/M same day as procedure or vaccine. Modifier 95 — telehealth via interactive audio+video; pair with POS 02 for parity reimbursement. Modifier LT/RT — laterality for joint injections.				
90472	Vaccine admin, add'l	—	\$16	Each additional vaccine
20610	Major joint injection	LT/RT	\$108	Knee/shoulder; + steroid J-code

CCM (99490/99439/99487) — recurring monthly billing; requires ≥2 chronic conditions + electronic care plan + 24/7 access. **AWV (G0438/G0439)** — HRA + fall risk + cognitive assessment required. **Vaccines** — both admin code (90471–90480) AND product code required, plus NDC.



FAMILY MEDICINE · DENIAL PREVENTION

Top 8 Family Medicine Denial Reasons & How Aethera Prevents Each

Family medicine denial rates average ~10% — lower than specialty averages, but high absolute volume makes the rework cost significant. CCM time documentation and AWW documentation gaps drive 40%+ of denials.

RED 22 %

CCM time documentation insufficient

99490/99439 billed without 20-minute time log supporting monthly threshold.

Aethera: CCM time-tracking platform with auto-billing at 20-min threshold + audit-ready time log.

RED 18 %

AWV documentation incomplete

G0438/G0439 billed without HRA + fall risk + cognitive assessment all documented.

Aethera: AWV template enforces HRA + fall risk + cognitive + depression screening before claim release.

AMBER 14 %

Modifier 25 misuse

E/M + procedure same day without separate documentation supporting E/M significance.

Aethera: Modifier 25 documentation checklist before claim release; auto-flag same-day combinations.

AMBER 11 %

Preventive + problem E/M same day

Both billed without modifier 25; reimbursement reduced or denied.

Aethera: Dual-visit rules engine auto-appends modifier 25 to problem E/M when AWW/preventive same day.

AMBER 9 %

Vaccine product + admin coding errors

Wrong vaccine product code or admin code (90471 vs 90472 ordering); NDC missing.

Aethera: Vaccine crosswalk table auto-selects correct admin + product code + NDC.

GREEN 7 %

Telehealth modifier 95 missing

POS 02 + modifier 95 required for parity reimbursement.

Aethera: Telehealth claim rules engine auto-appends modifier 95 + POS 02.

GREEN 6 %

TCM 30-day window violations

Billing TCM outside 30-day post-discharge window or for same patient twice in window.

Aethera: TCM tracker enforces single-claim-per-30-day-window per patient.

GREEN 5 %

BHI eligibility not verified

Patient without ≥2 chronic conditions for behavioral health integration.

Aethera: BHI eligibility check confirms ≥2 chronic dx + behavioral health care manager assignment.

CASE STUDY • INTEGRATED FM RCM

6-Physician Family Medicine Practice — Annual Revenue Impact

A \$3.8M-revenue practice models the combined upside of CCM enrollment, AWW + preventive capture, and telehealth parity enforcement.

Practice: **6 physicians** Annual revenue: **\$3.8M** Active patients: **18,000** Payer mix: **60% Medicare/Commercial**



1,000-patient Medicare panel eligible for CCM

Pre-Aethera enrollment: **12%** (120 patients)

Post-Aethera: **55%** (550 patients)

Recovered patients: **430**

Avg annual CCM per patient: ~ **\$975/yr** (99490 + 99439)

\$420,000

ANNUAL REVENUE



1,000-patient Medicare panel AWW-eligible

Pre-Aethera AWW completion: **35%**

Post-Aethera: **75%**

Recovered AWW visits: **400/year**

Avg AWW value: **\$185** (G0438) or **\$95** (G0439)

\$185,000

ANNUAL REVENUE



~ **3,000** telehealth visits/year

Pre-Aethera parity denial rate: **18%**

Post-Aethera: **5%**

Recovered telehealth claims: **390/year**

Avg telehealth E/M value: **\$92** (99213 parity)

\$135,000

ANNUAL REVENUE

Combined annual revenue recovery: \$740,000. Implementation cost (year 1): ~\$190K (Aethera RCM fees + CCM platform + AWW workflow + telehealth rules engine). Year-1 net: **\$550,000**.
ROI: **285%**.



FAMILY MEDICINE • COMPLIANCE HOTSPOTS

Where OIG, Medicare RAC, and Commercial Payers Audit First

Top compliance risks across the four family medicine pillars. Quarterly self-audits recommended.

✦ E/M + Acute	+ Preventive Care	{ Chronic Care Mgmt	+ Procedures & Vaccines
RED Modifier 25 misuse — appending to E/M when same-day procedure or vaccine does not represent significant, separately identifiable service; OIG Work Plan 2025-2026 focus area.	RED AWV documentation incomplete — billing G0438/G0439 without HRA + fall risk + cognitive assessment all documented; Medicare recoupment risk.	RED CCM time fraud — billing 99490/99439 without contemporaneous time logs totaling ≥20 minutes; OIG Work Plan 2025-2026 active audit target.	RED Vaccine product + admin coding errors — billing wrong vaccine product code or admin code; auto-denial + recoupment risk.
AMBER Telehealth without modifier 95 + POS 02 — billing telehealth at non-parity rate without proper modifier discipline.	AMBER Sports physical billed to insurance — 99212-99215 with Z02.0 billed to commercial; should be cash-pay. Refunds required if audited.	AMBER BHI eligibility not verified — enrolling patient in BHI without ≥2 chronic conditions documented; coverage violation.	AMBER Joint injection documentation missing laterality — billing 20600-20610 without LT/RT modifier; auto-denial.
GREEN E/M level over-coding — billing 99214/99215 without MDM documentation supporting the level; upcoding patterns trigger RAC audit.	LOW Preventive + problem E/M same day without modifier 25 — problem E/M denied; revenue loss only (no audit risk).	LOW TCM 30-day window violations — billing TCM twice in same 30-day post-discharge window or outside the window.	LOW Vaccine without NDC documented — vaccine claim without 11-digit NDC; auto-denial on Medicare + most commercial.

OIG Work Plan 2025-2026 explicitly lists CCM time logging, AWV documentation, and modifier 25 misuse as active audit targets. Medicare RACs have recovered **> \$240M** from primary care practices since 2021.



CLOSING · KEY TAKEAWAYS

Three Actions to Capture Family Medicine Revenue

For family medicine practice administrators and revenue cycle directors leading the next 90 days.

01

Stand up a CCM enrollment pipeline

Query EHR for patients with ≥ 2 chronic conditions (most FM patients qualify). Assign care coordinator to enroll 50 patients/week. Target 55% enrollment of eligible 1,000-patient Medicare panel. Generates **~ \$420K/year** of recurring monthly revenue at near-zero marginal cost — the single largest revenue line in family medicine.



02

Automate AWV documentation workflow

Build EHR template that enforces HRA + fall risk + cognitive assessment + depression screening before AWV claim release. Lift AWV completion from 35% to 75% on the 1,000-patient Medicare panel. Recovers **~ \$185K/year** in G0438/G0439 revenue with near-zero marginal cost.



03

Enforce telehealth modifier 95 + POS 02 discipline

Implement pre-submission rules engine that auto-appends modifier 95 + POS 02 on every telehealth claim. Telehealth parity denial rate drops from 18% to <5%. Recovers **~ \$135K/year** for a 6-physician practice running 3,000 telehealth visits/year at \$92 average E/M value.



Questions & Discussion

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